GlycoRenal Twin

Clinical report 16 Oct 2026 11:38

Patient IND-0042 | 54 y M | T2DM 9 years | Doctor: Dr. Anjali Mehta

Predicted risk

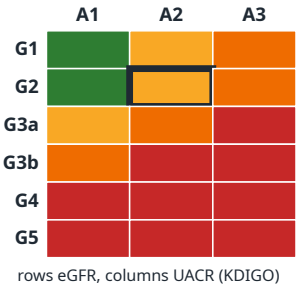
3-year risk of kidney disease worsening within 3 years: 35% (high).

Raising the risk: eGFR, Albuminuria (UACR).

Model: dkd_progression@2026-10-16

HbA1c forecast in 3 months: 8.3% (80% range 7.9 to 8.7%).

KDIGO G2-A2 (moderate risk). eGFR slope -8.02 per year; change over 12 months -11%.

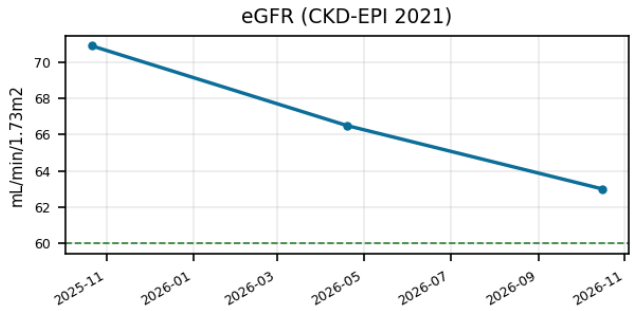


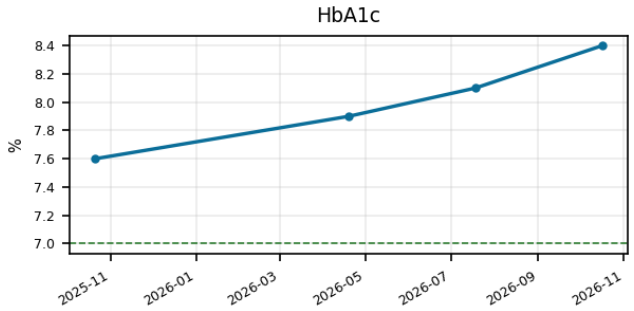
rows eGFR, columns UACR (KDIGO)

Latest results

Test	Value	Date	Age	Status
HbA1c	8.4 %	16 Oct 2026	0 days	current
Serum creatinine	1.34 mg/dL	16 Oct 2026	0 days	current
eGFR (CKD-EPI 2021)	63 mL/min/1.73m2	16 Oct 2026	0 days	current
Urine albumin/creatinine ratio	85 mg/g	16 Oct 2026	0 days	current
Potassium	4.6 mmol/L	16 Oct 2026	0 days	current
LDL cholesterol	118 mg/dL	16 Oct 2026	0 days	current
Blood pressure	146/88 mmHg	target < 130/80		above target
BMI / weight	27.9 kg/m2, 79.8 kg			
Medicine adherence	66.7% (7 d), 58.9% (30 d)			low

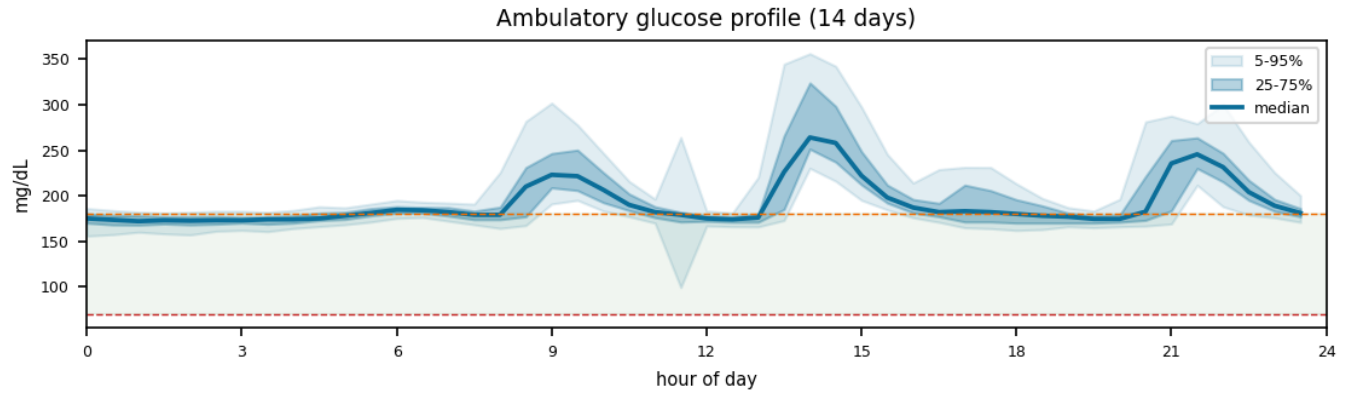
Trends





CGM summary (14 days)

Time in range 46.9% (target > 70%); below 70 mg/dL 0.1% (target < 4%); below 54 0.1%; above 180 53.0%; above 250 7.8%. Mean 193.8 mg/dL, GMI 7.95%, CV 17.4%, data sufficiency 97.3%.



Treatment options, ranked by predicted reduction in 3-year kidney risk

#	Option	3-yr risk	Evidence
1	Add GLP-1 receptor agonist (e.g. semaglutide)	35% to 25%	FLOW, Perkovic NEJM 2024 (verify)
2	Bring BP to target (< 130/80)	35% to 27%	KDIGO 2021 BP in CKD guideline (verify)
3	Add finerenone 10-20 mg	35% to 28%	FIDELIO-DKD, Bakris NEJM 2020 (verify)
4	Start basal insulin	35% to 33%	ADA Standards of Care 2026 sec 9 (verify)
5	Add sulfonylurea (e.g. gliclazide)	35% to 34%	ADA Standards of Care 2026 sec 9 (verify)
6	Add DPP-4 inhibitor (e.g. linagliptin)	35% to 35%	ADA Standards of Care 2026 sec 9 (verify)
7	Lose 5% of body weight	35% to 35%	Look AHEAD, Wing Diabetes Care 2011 (verify)
8	Walk 2,000 more steps a day	35% to 35%	Umpierre JAMA 2011 (verify)

Combined plan (Add GLP-1 receptor agonist (e.g. semaglutide), Add finerenone 10-20 mg, Bring BP to target (< 130/80)): 35% to 16%.

Not eligible or already in place: Add SGLT2 inhibitor (e.g. dapagliflozin 10 mg) (Already on an SGLT2 inhibitor (continue until dialysis/transplant)); Start / maximise ACE inhibitor or ARB (e.g. telmisartan) (Already on ACEi or ARB: titrate to max tolerated dose; never combine ACEi with ARB); Add metformin (Already on metformin); Stop smoking (Non-smoker)

Decision support only. Suggestions with sources; the treating clinician decides.

Current medicines

Metformin 1000 mg at 08:00, 20:00; Telmisartan 40 mg at 08:00; Dapagliflozin 10 mg at 08:00

Active alerts

[critical] Glucose 52 mg/dL (below 54): severe hypoglycaemia
[warning] Glucose 64 mg/dL (below 70): hypoglycaemia
[warning] Low glucose likely within 30 min (now 126, forecast 133, trend 78 mg/dL)
[warning] Albuminuria rose from A1 to A2 (UACR 85 mg/g)
[warning] Glucose 260 mg/dL (above 250)
[warning] Medicine adherence 67% in the last 7 days

Clinical summary (offline template)

IND-0042, 54M, T2DM 9 yrs. HbA1c 8.4% (change 1 yr 0.8), eGFR 63 (slope -8.02/yr), UACR 85 mg/g, KDIGO G2-A2 (moderate). BP 146/88 (target < 130/80). TIR 47%, adherence 67% (7 d).

Predicted 3-year risk of kidney disease worsening within 3 years: 35% (high); main drivers: eGFR, Albuminuria (UACR). HbA1c forecast in 3 months: 8.3%.

Options ranked by predicted risk reduction:
1. Add GLP-1 receptor agonist (e.g. semaglutide): DKD 3-yr risk 35% to 25%; HbA1c 8.4% to 7.4% (source: FLOW)
2. Bring BP to target (< 130/80): DKD 3-yr risk 35% to 27% (source: KDIGO 2021 BP in CKD guideline)
3. Add finerenone 10-20 mg: DKD 3-yr risk 35% to 28% (source: FIDELIO-DKD)
4. Start basal insulin: DKD 3-yr risk 35% to 33%; HbA1c 8.4% to 6.9% (source: ADA Standards of Care 2026 sec 9)

Decision support only. Suggestions with sources; the treating clinician decides.